



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : هناء عبد الحافظ احمد ابوريه
المحافظة : الجيزة
الحى / المدينة : ابوالنمرس
رقم البطاقة : 28102102104288 رقم التليفون : 01128244588
العنوان : ترسا
العمر (تاريخ الميلاد) : 10-02-1981 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هناء عبد الحافظ احمد ابوره National ID: 28102102104288	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٥/٠٦ ٢٨١.٢١.٢١.٤٢٨٨

ربة منزل



أنثى مسلمة
حماده خليل حميد حسن

البطاقة سارية حتى ٢٠٢٢/٠٦/٢٧



جمهورية مصر العربية
بطاقة تحقيق الشخصية

هنا

عبدالحافظ احمد ابوريه

ترسا

مركز ابو النمرس - الجيزه

١٩٨١/٠٢

٢٨١.٢١.٢١.٤٢٨٨

LF7316892

03/26/2026

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MDT FORM

Name:	Henaa Abdelhatez Ahmed Abi Raya	MRN:	8841
Age:	45 Y	ID:	28102102104288
Menopausal Status:	premi	Phone:	01128244588
NRS SCORE:	ps 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason OF MDT:	

Clinical Data:

BX:

loc triple -ve

R

ER: 2/8

PR: 2/8

HER2: +1

KI67: 80%

L

Sonemammography:

Mets Workup:

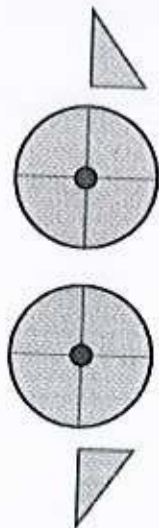
History:

premi left breast cancer ct2 n2 mo by cts triple -ve ki 80% on exam left uq mass 2.5x2 left axil in 2x3 .cts free sm uq mass and intramammary in path ax in for 2 clips for new labs echo bs to start taxol carboplatin after approval for remat to add peritrozolunab

Recommendations:

MDT Outcome & Plan

For Bone Scan - ECHO
after Clipping to Start Pemtro-Taxel-Carbo ---then Pemtro- AC then surgical evaluation
BRCA test



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شفاء ءمؤف / د/ شفاء ءمؤف	د/ شفاء ءمؤف	د/ شفاء ءمؤف			

Patient Name : الأستاذ / هناء عبد الحافظ احمد	Request No. : 13003777
Patient No. : 28102102104288	Request Date : 03/12/2025
Path No. : 3748/25	Verification Date : 10/12/2025
Sex : Female	Financial Category : جوستاف روسي - مبادرة
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical data

Left breast UIQ mass

Nature of specimen

Tru cut biopsy, collected on 3/12/2025, on 1:00, cold ischemia time :10 minutes .

Gross examination

Two grayish white tissue cores measuring 0.5 cm and 1 cm, totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

P63 (4A4)

Microscopic examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 3.
- * DCIS: Absent
- * Angioinvasion: Absent
- * Perineural invasion: Absent

IPT :

P63 :Negative .

Diagnosis

Left breast UIQ mass, Tru cut biopsy:

Invasive duct carcinoma, Grade 3.

For further IHC (ER, PR, HER2, Ki67)

Verified by :

Dr.Heba Hasan

Printing Date : 14/12/2025 12:16:50

Page 1 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / هناء عبد الحافظ احمد	Request No. : 13003777
Patient No. : 28102102104288	Request Date : 03/12/2025
Path No. : 3748/25	Verification Date : 10/12/2025
Sex : Female	Financial Category : جوسفان روسي - ميانره
Age : 44 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Weak nuclear reaction in less than 1% of tumor cells (Allred score 2/8).
- PR: Weak nuclear reaction in less than 1 % tumor cells (Allred score 2/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 80%

Conclusion:

Left breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade 3.
IPT:

- ER: Negative (score 2/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).
- Ki67: 80 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :
Dr.Heba Hasan

Printing Date : 14/12/2025 12:16:54

Page 1 of 2

Printed By : .marwa mohamed yousse



Patient Name: الأستاذ / د. عبد الحافظ أحمد
Patient No.: 28102102104288
Path No.: 3748/25
Sex: Female
Age: 44 Y
Referring Doc.

Request No.: 13003777
Request Date: 03/12/2025
Verification Date: 10/12/2025
Financial Category: جومات روسي - مبادرة
Department: Out Patient

Registered by: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Choneim
Dr. Heba El-Shafeey

Verified by :
Dr. Heba Hasan

Printing Date : 14/12/2025 12:16:54

Page 2 of 2

Printed By : marwa mohamed yousse



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Name : hanaa abd al hafiz cap+c

Branch : Portable

Ref.Physician Prof :

Date : 31-12-2025 02:20

Medical No. : GRI-28102102104288

Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Known patient of left breast cancer& ipsilateral pathological LNs

1-Chest:

- No sizable pulmonary nodules, masses, cavitary lesions or bronchiectasis changes.
- Right middle lobe atelectatic bands
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2-Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No focal lesions
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

Name : hanaa abd al hafiz cap+c

Branch : Portable

Ref.Physician Prof :

Date : 31-12-2025 02:20

Medical No. : GRI-28102102104288

Ins.Company :

- Normal appearance of the pancreas, aorta, I.V.C and supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Divarication of recti

3-The scanned skeleton:

- Left acetabulum sclerotic focus likely bone island
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- No definite hepatic, pulmonary or osseous metastatic deposits.

DR/NERMEEN RAMADAN.MD

DOC GRI

DOC GRI

Patient ID: 94931	Patient Name English: Henea Abd Alhatez Ahmed
Age: ٤٥ سنة ١ شهر	2025/04/22 12:05:37 PM Report Date

Women's Health Outreach Program
Ultrasound guided clips insertion

Left breast

- Under complete aseptic condition & ultrasound guidance two clips insertion were done for left breast pathologically proven malignant mass located at 12:00 and pathological IMN located at 1:00. The clips were inserted at the center of both lesions and this confirmed by mammographic CC & MLO views

Best Regards,
Dr. Eman Ali

Name : Hanaa Abdel Hafiz Ahmed Aborieh
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 09-04-2026 12:57
Medical No. : 346660
Ins.Company : شركات مستشفيات

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female, 45 years old; history of breast carcinoma. Evaluation for possible skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time. Additional spot views were obtained when necessary.

Findings:

- o No focal abnormal areas of increased or decreased tracer uptake are identified to suggest skeletal metastatic deposits on scintigraphic basis.
- o Expected age-related degenerative changes may be seen (at different levels of the spine and around big joints) without suspicious metastatic pattern.

IMPRESSION:

- o No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- o Follow-up imaging only if clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

ECHO REPORT

NAME: سید بلال

ID NUMBER: /

DATE: 3 / 5 / 2026

SEX: AGE:

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDS	2.3	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GE

SWMA No RUMA

LA and Ao Root Dimensions Normal

Mitral Valve Normal

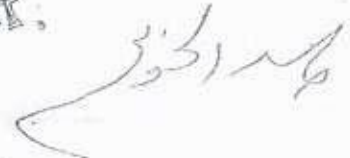
Aortic Valve Normal

Tricuspid Valve Normal

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.3

Intracardiac Masses and Effusion

DR. 

Patient ID: 93513	Patient Name English: Hanan / Abdalhafez Ahmed
عمر: 44	Aboraih
	2026/02/22 11:01:09 AM Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 44 years old female patient with pathologically proven IDC grade III (-ve ER, -ve PR, -ve HER2 and Ki67= 80 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild nodular background enhancement

ACR: B (scattered fibro-glandular densities)

LEFT BREAST:

- Low energy (Non contrast) images: The left breast shows circumscribed dense mass with partially obscured margins at upper central measures about 2.3x1.7cm and about 4.7 cm from the nipple.
- Recombined (Post contrast) images: The left breast shows moderate heterogeneous upper central mass enhancement measures about 3x2x2 cm at its AP, TS and CC diameters and about 5.4 cm from the nipple.
- Normal skin / nipple / areolar complex.
- Complementary US:
 - The left breast shows rather defined hypoechoic mass lesion located at 12 ° clock position and measures about 2x1.4 cm and about 3 cm from the nipple with posterior linear extension measuring about 7 mm
 - A single enlarged axillary lymph nodes at level I. with lost hilum measuring about 4.5x2 cm

RIGHT BREAST:

- Low energy (Non contrast) images No masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images show no pathological uptake.
- Normal skin / nipple / areolar complex.
- Complementary ultrasound:
 - Few enlarged axillary lymph nodes with diffuse cortical thickening

IMPRESSION:

LEFT BREAST: (BIRADS 6= pathological proven) Left upper central unifocal pathologically proven IDC with ipsilateral pathological axillary lymph node as described above

RIGHT BREAST: (BIRADS 2= benign) Normal study

Best regards,

Dr. Norhan osama, MD

1111

Patient ID: 90325	Patient Name English: Hanaa Abdelhaleem Ahmed
Age: 45	2025/1/26 11:40:58 AM Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 45-year-old lady with family history complaining of left axillary pain and lump of few weeks duration.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- UIQ, middle third, irregular equal density mass lesion with partially obscured margins and surrounding subtle architectural distortion.
- No suspicious clusters of micro calcifications.
- Ultrasound demonstrated 11-12 o'clock, middle third, 5 cm from nipple, 13 mm deep to the skin, fairly oval parallel mass lesion yet showing angulated/micro-lobulated outlines heterogenous echogenicity and surrounding subtle increased fat lobules echogenicity measuring 15 x 10 mm; it showed minimal internal vascularity on color Doppler study.
- UOQ, posterior third, 1-2 o'clock pathological intra-mammary lymph node measuring 9 x 7 mm.
- Normal skin and nipple/areolar complex.
- Solitary level 1 pathological axillary LN is noted measuring 12 x 7 mm. No pathological infra or supra-clavicular lymph nodes.

Right breast

- Scattered fibro glandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast (BIRADS 4c): UIQ suspicious solid mass lesion with pathological intra-mammary and solitary level 1 axillary pathological lymph nodes.

Right breast (BIRADS 1): Normal study.

Recommendations:

Name : Hanaa Abdel Hafiz Ahmed Aborieh
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 09-04-2026 12:57
Medical No. : 346660
Ins.Company : تعاليد مستشفىفات

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female, 45 years old; history of breast carcinoma. Evaluation for possible skeletal metastases.

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- o Expected age-related degenerative changes may be seen (at different levels of the spine and around big joints) without suspicious metastatic pattern.

IMPRESSION:

- o No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- o Follow-up imaging only if clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

ECHO REPORT

NAME:

56/1

DATE:

15/9/2021

ID NUMBER:

1850/66

SEX:

♀

AGE:

EF	55-75%	PWT	0.6-1.1cm
.VEDD	3.5-5.8cm	AO.R	3-3.7cm
.VESD	2-4cm	LA	2.4cm
.S	25-45%	RV	0.7-2.3cm(supine)
PWT	0.6-1.1cm	AO.CUSP	1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

study

10/10/21